

VirtualWorks Internship – Task 5

End-to-End Individual Case Safety Report (ICSR) Processing

SIMULATED CASE NOTICE

This is a fictional case created solely for educational and internship-project purposes. It does not represent a real patient, real safety report, or actual regulatory submission.

1. Case Information

Field	Value
Case ID	VW-T5-001
Patient	45-year-old female
Reporter	Treating physician
Suspect drug	Diclofenac 50 mg tablets
Indication	Acute musculoskeletal pain
Adverse event	Upper gastrointestinal bleeding (melena)
Therapy start	2026-08-08
Event onset	2026-08-14
Therapy stop	2026-08-14
Medical history	Osteoarthritis; no documented previous gastrointestinal bleeding
Concomitant medication	Omeprazole 20 mg once daily
Outcome	Recovered
Dechallenge	Positive
Seriousness	Serious – hospitalization
Severity	Severe
Causality	Probable
Reporting category	15-day

2. Assessment & Workflow

Triage: Valid simulated ICSR.

The exercise represents the four core validity elements.

ADR identification: Upper gastrointestinal bleeding (melena).

The event follows exposure to diclofenac.

Seriousness: Serious – hospitalization.

Hospitalization is the seriousness criterion used here.

Severity: Severe.

Severity is treated separately from seriousness.

Causality: Probable.

the simulated chronology, the known association between NSAID exposure and gastrointestinal bleeding, positive dechallenge, and absence of a stronger alternative explanation in this constructed case, the causality is assessed as probable.

Dechallenge: Positive.

The event resolved after discontinuation and treatment.

Reporting category: 15-day.

Based on hospitalization.

3. Case Narrative

A 45-year-old female with osteoarthritis was treated with diclofenac 50 mg tablets for acute musculoskeletal pain beginning 08-Aug-2026. On 14-Aug-2026, she developed melena and was hospitalized for evaluation and treatment of upper gastrointestinal bleeding. Diclofenac was discontinued on the same day. The patient received appropriate supportive treatment and the event resolved. Omeprazole 20 mg once daily was reported as a concomitant medication. Based on the simulated chronology, the known association between NSAID exposure and gastrointestinal bleeding, positive dechallenge, and absence of a stronger alternative explanation in this constructed case, the causality is assessed as probable.

4. Quality Review

Structured fields, assessments, timeline and narrative were cross-checked for internal consistency.

5. Methodology Note

The tutorial describes triage, data entry, quality review, medical review and submission. It does not specify a particular causality scale or severity grading scale. The classifications in this simulated case are therefore presented as an educational exercise, not as values taken directly from the tutorial.

6. Disclaimer

All patient, reporter, chronology and clinical details are fictional. This project is not an actual ICSR and must not be submitted to a regulatory authority.